



Statement of Work (SOW)

Project Title: Upgrade Interior Signage and Wayfinding

Project Number: 548-23-105

Veterans Administration Health Care System (VAHCS)

West Palm Beach

7305 N. Military Trail, West Palm Beach, FL 33410

Revision 1

March 24, 2026

Facilities Management Services

1	Overview	3
2	Definitions and Acronyms	4
3	Scope of Work.....	6
	3.1 Areas of Work	Error! Bookmark not defined.
	3.2 Protection of Existing Work.....	7
	3.3 Changes to the Statement of Work.....	7
4	Scheduling, Phasing, and Working Hours	8
	4.1 Schedule of Contract	8
	4.2 Performance Period.....	8
	4.3 Phasing of Construction	8
5	Infection control and Safety Measures	9
	5.1 Interim Life Safety Measures Evaluations (ILSM).....	9
	5.2 Infection Control Risk Assessment (ICRA).....	9
	5.3 Pre-Construction Risk Assessment (PCRA).....	11
6	Construction Requirements.....	12
	6.1 Quality Control Plan:	12
	6.2 Safety	122
7	Security Requirements.....	13
8	Asbestos Management Program.....	13
	8.1 Purpose.....	13
	8.2 Facility Asbestos O&M Plan.....	14
	8.3 Pre-Construction Asbestos Surveys.....	14
	8.4 Policy.....	15
9	Reporting Requirements	15
	9.1 Progress Reports.....	15
	9.2 Progress Meetings	15
10	Government Furnished Information	15
11	Resources.....	16
	11.1 VA Standards and Publications.....	16
12	Confidentiality and Nondisclosure	16
	Appendix A – Drawings	17

1 Overview

This project is a redesign-upgrade of the existing wayfinding, identification, and regulatory interior signage for 548-23-105 Upgrade Interior Signage and Wayfinding at the West Palm Beach VA Medical Center, West Palm Beach, Florida. The documents included in the SOW are below but are not limited to:

- 05 Statement of Work 548-23-105
- 07 Specifications
- 07a SOW Narrative
- 07b Drawings

West Palm Beach VAMC Project # 548-23-105 Upgrade Interior Signage and Wayfinding at the West Palm Beach VA Medical Center, West Palm Beach, Florida.

2 Definitions and Acronyms

- **Asbestos:** Asbestos is a generic term for a number of asbestiform hydrated silicates that, when crushed or processed, separate into flexible fibers made up of fibrils. Asbestos means chrysotile, cummingtonite-grunerite asbestos (amosite), crocidolite, anthophyllite asbestos, tremolite asbestos, and actinolite asbestos.
- **Asbestos Containing Material:** Any material containing more than 1 percent asbestos. ACM can be divided into three major categories - thermal system insulation, surfacing and miscellaneous.
- **Contracting Officer (CO):** The services to be performed under this contract are subject to the general supervision, direction, control and approval of the Contracting Officer.
- **Project Manager (PM)/Contracting Officers Representative (COR):** The Contracting Officer's representative responsible for administering contracts under the immediate direction of the Contracting Officer.
- **Contractor:** This term, as used herein, refers to the contractor under this contract.
- **A/E:** This term, as used herein, refers to the Architect Engineer firm(s) that are a part of the DB team, also referred to as DB A/E.
- **Pre-Construction Risk Assessment (PRCA):** Used to identify potential risks associated with the renovation, construction, and some maintenance & repair activities and to develop risk mitigation strategies to minimize these risks. Prior to beginning work this assessment will be completed.
- **Interim Life Safety Measures (ILSM):** Measures that are instituted to compensate for significant hazards posed by existing life safety (fire) code deficiencies or construction/renovation activities.
- **Request for Proposal (RFP):** Is a solicitation made, often through a bidding process, by an agency or company interested in procurement of a commodity or service, to potential suppliers to submit proposals.
- **Certified Independent Third-Party Safety Professional (CITP):** Certified Independent Third-Party Safety Professional for the review of the design documents for compliance with national and local codes, standards, federal and state regulations.
- **National Fire Protection Association (NFPA):** The world's leading advocate of fire prevention and an authoritative source on public safety, NFPA develops, publishes, and disseminates codes and standards intended to minimize the possibility and effects of fire and other risks.
- **The Joint Commission (TJC):** The Joint Commission provides evaluation, accreditation, and disease-specific certification programs for healthcare facilities.
- **Competent Person (CP):** One who is capable of identifying existing and predictable hazards in the surroundings or working conditions which are unsanitary, hazardous, or dangerous to employees, and who has authorization to take prompt corrective measures to eliminate.
- **Facility Asbestos Operations and Maintenance Program:** The principal objective of a Facility Asbestos O&M program is to minimize exposure of all building occupants to asbestos fibers. To accomplish this objective, an O&M program includes work practices to (1) maintain ACM in good condition, (2) ensure proper cleanup of asbestos fibers

previously released, (3) prevent further release of asbestos fibers, and (4) monitor the condition of ACM.

3 Scope of Work

The contractor will provide all labor, materials, tools, and associated equipment necessary for the installation and delivery of the service for the areas of work of this project as listed below. All proposals should include all stages of services for this project. The respective areas of work are shown in the enclosed **07 Specifications, 07a SOW Narrative, 07b Drawings**. They are, but may not be limited to the following:

3.1 Work includes but not limited to:

- 3.1.1 West Palm Beach VAMC Project # 548-23-105 Upgrade Interior Signage and Wayfinding at the West Palm Beach VA Medical Center, West Palm Beach, Florida.
The facility has recently deployed some room identity and wayfinding signage in various areas of the facility. Stakeholders have requested that this project use the recently deployed signage as the aesthetic benchmark for the new interior signage project. The communicated desire is to retain the recently deployed signage where practical. It has been observed that several of the recently deployed signs are installed at heights/positions that are not ADA-compliant and the VA would like to rectify these compliance issues as part of this project. To this end, the following strategies are proposed:
In spaces with older signs, the older signs will be specified for removal and new signs specified for deployment.
In spaces with newer signs, they will be evaluated for ADA-compliance and necessary corrective action will be specified as needed.
Existing facility directories will be specified for replacement with digital non-interactive directories.
Existing overhead directional signs will be specified for replacement as needed.
Newer wall-mounted eye-level directional signs will be evaluated for ADA-compliance and compatibility with the new signage design and necessary adjustments will be specified.
See 07a SOW Narrative for more specifications.
- 3.1.2 *Close out Documentation* – Contractor is responsible for providing locations indicated on facility floor plan drawings of all equipment and cabling associated with the new Upgrade Interior Signage and Wayfinding.
- 3.1.2 *Quality Control* – Contractor is responsible for maintaining a clean environment during the installation of the equipment. Upon completion of the project all remaining materials are to be removed or disposed of and existing equipment shall be properly turned in through property management. All codes shall be strictly adhered to and maintained throughout the entirety of the project and upon completion a final inspection will be done to verify codes, standards or regulations have been violated.

3.1.3 *Working Hours* – Working hours shall be between 8am. and 6pm Monday through Friday. Contractors are required to check in and out with the POC each day. (1) Some areas may be inaccessible during normal working hours and may need to be done after hours. Contractor shall include that in their proposal as additional funding will not be available. (2) After hours work is required, a request will be submitted to the COR before starting work for that request to allow time to coordinate access. Contractor will need to schedule around required delays by the hospital staff and may have to repeat attempts to complete these areas. These delays shall be coordinated with the Medical Center POC.

3.1.4 Contractor shall be responsible for any patching/painting/caulking around devices or damaged areas during installation of new devices/systems.

3.2 Protection of Existing Work

3.2.2 The Contractor shall preserve and protect all structures, equipment, and vegetation (such as trees, shrubs, and grass on or adjacent to the work site and which do not interfere with the work required under this contract. If any limbs or branches are broken during the contract performance by careless operation of equipment or employees, the contractor shall clean cut and paint the cut with a tree-pruning compound approved by the COR.

3.2.3 The Contractor shall protect from all damage of improvements and utilities at or near the work site and on adjacent property including those that are the property of a third party from failure to comply with the requirements and reasonable care while performing the work.

3.2.4 The contractor shall under regulations prescribed by the Contracting Officer to use only the established approved roadways. When materials or equipment are transported, vehicles must not exceed loading capacity recommended by manufacturer and local or state laws to include the load limit on the upper parking garage deck. When it is necessary for the contractor to cross curbs or sidewalks the contractor shall protect them from damage.

3.2.5 It is the responsibility of the contractor to repair suitably or replace all damaged property due to contractors' negligence.

3.3 Changes to the Statement of Work

3.3.2 Any changes to this SOW shall be authorized and approved only through written correspondence from the CO. A copy of each change will be kept in a project folder along with all other products of the project. Costs incurred by the contractor through the actions of parties other than the CO shall be borne by the contractor.

4 Scheduling, Phasing, and Working Hours

4.1 Schedule of Contract

- 4.1.1 Contractor shall submit a schedule with estimated dates and itemized cost of work to the Contracting Officer for review and approval before Notice to proceed.
- 4.1.2 The contractor will be required to provide the Contracting Officer with a comprehensive phasing plan to perform this contract. The contractor will be required to update this plan on a weekly basis to provide the VAMC management team with an accurate status of the work. A two-week notice to the Contracting Officer will be needed for approval prior to acceptance and authorization of all changes to the project schedule.

4.2 Performance Period

- 4.2.1 The contractor shall complete the work required under this scope in **270** calendar days or less from date of award. Extensions may be granted as directed by project VAMC management team. If the contractor proposes an earlier completion date, and the Government accepts the contractor's proposal, the contractors proposed completion date shall prevail.
- 4.2.2 Work at the West Palm Beach VAHCS site shall take place between the hours of 8am until 6:00pm Monday through Friday or as approved by the COR. Contractors are required to check in and out with the POC each day. (1) Some areas may be inaccessible during normal working hours and may need to be done after hours. Contractor shall include that in their proposal as additional funding will not be available. (2) After hours work is required, a request will be submitted to the COR before starting work for that request to allow time to coordinate access. Contractor will need to schedule around required delays by the hospital staff and may have to repeat attempts to complete these areas. These delays shall be coordinated with the Medical Center POC.
- 4.2.3 The contractor shall not commence performance of the tasks in this scope until the VAMC has conducted a kickoff meeting or has advised the contractor that such a meeting has been waived.

4.3 Phasing of Construction

- 4.3.1 The contractor will be required to provide the Contracting Officer a comprehensive schedule to perform this project. A two week notice to the Contracting Officer will need to be approved prior to any changes affecting the schedule.
- 4.3.2 Four (4) Days' notice is also required for any closures, disruptions installation, etc. that will impact normal daily operations such as blocking off areas. Utility Outages, including disruption of fire alarms, must be requested by the Contractor at least 4 calendar days in advance.

4.3.3 During the construction phase the Contractor shall perform all work during the hours of 8am until 6:00pm Monday through Friday.

4.3.4 All submittals will be returned to the Contractor within two (2) weeks of submission. Disapproved submittals are not reason for additional time or delay claim for the Contract.

5 Infection control and Safety Measures

5.1 Interim Life Safety Measures Evaluations (ILSM)

5.1.1 The contractor and COR will be responsible to identify areas in their design that will require Interim Life Safety Measures in accordance with the latest - The Joint Commission (TJC) Hospital Standard LS.01.02.01. The contractor will clearly define areas requiring Interim Life Safety Measures and the necessary precautions needed. Along with the requirement for Interim Life Safety Measures, the contractor will work with the COR and Safety Department to identify the needs, as well as identify the responsibilities of the Interim Life Safety Measures. Interim Life Safety Measures Evaluations will be completed for every project, with considerations made for all locations and phases.

5.2 Infection Control Risk Assessment (ICRA)

5.2.1 To establish policies and procedures to ensure the Safety & Environment of Care in all patient care buildings is not compromised during any renovations or new construction. When planning demolition, construction or renovation work, the contractor, Engineering, Infection Control & Safety personnel will conduct a proactive Pre-Construction Risk Assessment (PCRA) using risk criteria to identify hazards that could potentially compromise patient care in occupied areas of the building. The Infection Control Risk Assessment will be completed for every project, with considerations made for all locations and phases. The risk criteria should address the impact of demolition, renovation or new construction activities on Air and Water Quality requirements, Infection Control, Utility Requirements, Noise, Vibration and Emergency procedures.

5.2.2 Appendix A: Alternative to Outdoor Exhaust Requirements:

This Appendix provides an equivalent ICRA control measure to requirements detailed in the VHA ICRA template (Table 5 Level III, item 5) that would allow interior construction space exhaust.

Interior Construction Space Exhaust:

If the facility is not able to exhaust to the building exterior, the Healthcare Environment and Facilities program office has identified an alternate industry-recognized standard interior exhaust procedure that is accepted as providing an equivalent level of infection prevention and control (IPC), and safety. The alternate interior exhaust procedure is implemented by completing the following:

- a. If exhaust is directed indoors, then the system must be HEPA filtered. Prior to start of work, including assembly of the ICRA barriers, initial baseline air sampling measurements for particulate levels must be taken in the planned area of discharge. Once containment is erected and completed, HEPA filtration must then be verified by particulate measurement, using a particle counter with multi-channel capabilities to measure down to 0.3 microns, to confirm that the discharge of the HEPA unit does not exceed the baseline particulate count of the area to which the exhaust is directed. Verification must continue at least three times per day (once every 8 hours), 7 days per week, during the use of the negative air machine and HEPA filter. Any exceedance must trigger a work stoppage and an immediate response and resolution. All particulate measurements, including the baseline measurement, must be documented and retrievable.
- b. To accommodate the sampling, the negative air pressure machine must be fitted with a 2 ft section of ridged ductwork at the discharge. The sample port for monitoring must be in the ridged ductwork section.
- c. Vapor generating processes must be assessed and appropriate filtration must be installed to mitigate transmission to the facility.
- d. Engineering must do a complete flow and pressure analysis and document the area to which the air is being discharged to ensure the flow and pressure relationships in that area are not being adversely impacted by the additional flow. Ideally, this is done as part of the construction package to ensure the design is implementable and no additional HVAC changes are required to accommodate the exhaust from the negative air equipment.
- e. Install device on exterior of work containment to continually monitor negative pressurization. To ensure proper pressure is continuously maintained, the device(s) shall have a visual pressure indicator, data collection and alarm.
- f. Exhaust must be directed to Low-Risk areas as defined in Table 3 in the VHA ICRA template. Ideally this discharge must be located so as not to impinge on high traffic areas or create a nuisance with noise or excessive airflow.

NOTE: Exhaust into shared or recirculating HVAC systems, or other shared exhaust systems (e.g., bathroom exhaust) is not acceptable.

5.3 Pre-Construction Risk Assessment (PCRA)

5.3.1 The PCRA is made up of 13 individual assessments to be performed by subject matter expert(s) (SME), as identified in Service-specific procedures for the execution of PCRA(s) for which a Service is responsible. All 13 assessments in the PCRA will be completed for every project, with considerations made for all locations and phases. The 13 assessments that make up PCRA and the Service and SME responsible for performing the assessment are listed in the table, Service(s) and SME(s) Responsible for a Portion of PCRA

6 Construction Requirements

6.1 Quality Control Plan:

- 6.1.1 The contractor shall prepare establish and maintain an effective quality control (QC) system. QC consists of plans, procedures, and organization necessary to produce an end product which complies with the Contract requirements.
- 6.1.2 The project superintendent will be held responsible for the quality of work and is subject to removal by the Contracting Office or Authorized designee for non-compliance with the quality requirements specified in the Contract.

Submittals and Shop Drawings

- 6.1.3 A submittal is information submitted for VA review to establish compliance with the contract documents.
- 6.1.4 Submittals are to be scheduled, submitted, reviewed, and approved prior to the acquisition of the material or equipment. Coordinate scheduling, sequencing, preparing, and processing of submittals with performance of work so that work will not be delayed by submittal processing. Allow time for potential resubmittal.
- 6.1.5 All submittals are required to be approved prior to the start of the specified work activity.
- 6.1.6 All submittals will be returned to the Contractor within two (2) weeks of submission. Disapproved submittals are not reason for additional time or delay claim for the Contract.

6.2 Safety

- 6.2.1 Contractors' personnel are required to have completed up to date OSHA-approved 10-hour construction worker course. The onsite general superintendent/supervisor is required to have completed up to date 30-hour OSHA-approved construction course prior to working on site.
- 6.2.2 Contractor to provide the Contracting Officer with a copy of the OSHA certificates prior to executing work for the Veterans Administration. In addition, Contractor shall provide submittals for contract construction or renovation work which includes the names, qualifications, and training dates for Contractors Competent Persons (CP), who has been designated to administer the site-specific safety program as well as the CP for all other activities as required by OSHA regulation 29 CFR 1926.32 (such as scaffolds, cranes, etc. The VA's CP does not take the place of Contractors CP nor act in its behalf. The VA CP determines if the Contractor is meeting the VA standards and contractual requirements for safety and OSHA compliance. When these standards and contract requirements are not being met, the VA COR and or CP will take immediate action to prevent injury, non-compliance, and or property damage and the Contracting Officer will be notified immediately.

The Contractor shall submit an Activity Hazard Analysis and an Accident Prevention Plan as per FAR 52.236-13 prior to commencement of site activities. The government Shall have 21 calendar days to review the AHA and APP. Contractor is to provide Safety personnel (flagman), safety signs, cones, etc. during any activity within the of the normal traffic pattern or when deemed necessary by the COR.

6.2.3 Contractor employees shall be required to maintain their appearance, including a shirt bearing a company logo.

6.2.4 Contractor to adhere to the latest COVID precautions as directed by the VA Medical Center West Palm Beach.

7.1 Security Requirements:

7.1.1 Prior to granting access to any work area (i.e., building, area, room, or space) for mold remediation work, a determination must be made by the government agency whether classified or controlled unclassified information (paper material or electronic media) or equipment is contained in the work area(s). It may be necessary depending on the sensitivity of the work area or the information contained in the area to authorize the Government activity or tenant command responsible for the work area to provide their own appropriately cleared VA or government personnel to properly remove or secure any classified or controlled unclassified information, electronic media or equipment located in their work area(s). Prior authorization would be required, and the area would need to be evaluated to ensure it is safe for personnel to enter and all personnel must utilize the required PPE to safely enter the work area.

7.1.2 If Contractor personnel require access to classified information or spaces to perform mold remediation work, the Government must issue the Contractor facility a Facility Clearance Level (FCL) (Contract Security Classification Specification) prior to the initiation of the work under the contract. If the Contractor facility does not possess a valid FCL issued by the Defense Security Service (DSS), the Government will be required to submit a sponsorship request to DSS requesting that the Contractor be processed for and issued a current FCL at the appropriate level.

8 Asbestos Management Program

8.1 Purpose

8.1.1 This directive establishes the Veterans Health Administration (VHA) policy on the identification, management, and control of hazards related to asbestos-

containing materials (ACM) at Department of Veterans Affairs (VA) facilities. It is the intent of this policy to assist facilities in developing a comprehensive building asbestos survey, Operations and Maintenance (O&M) plan, and local policies that comply with each of the federal regulations listed below. AUTHORITY: Title 38 United States Code 7301(b); 29 Code of Federal Regulations (CFR) 1910.1001; 29 CFR 1926.1101; 40 CFR Part 763, Subpart E; and 40 CFR 61.

8.2 Facility Asbestos O&M Plans

- 8.2.1 Facility Asbestos O&M Plans are required for the ongoing management of ACM and shall include provisions for comprehensive building asbestos surveys, asbestos removal and other response actions associated with renovations involving ACM. They are the written permanent record of the current status and condition of all ACM in a facility's inventory. The Facility Asbestos O&M Plan provides the documentation for all asbestos management efforts, and procedures for overseeing the entire facility asbestos management program, including procedures to ensure compliance with applicable OSHA, EPA, and state and local regulations.
- 8.2.2 The Facility Asbestos O&M Plans must communicate how the VA medical facility will manage three types of projects:
 - 8.2.2.1 Those that are unlikely to involve any direct contact or disturbance with ACM;
 - 8.2.2.2 Those that may cause accidental disturbance of ACM; and
 - 8.2.2.3 Those that will involve small-scale, short-duration disturbances of ACM.
- 8.2.3 Facility Asbestos O&M Plans must be site-specific and tailored for all buildings. The plans should consider, use, function, and design characteristics of a particular building. At a minimum the plan must address:
 - 8.2.3.1 Organizational structure and assigned responsibilities for carrying out asbestos related work.
 - 8.2.3.2 Response action capabilities and procedures.
 - 8.2.3.3 Worker protection and supply requirements.
 - 8.2.3.4 Project coordination and communication necessary for minor projects, construction and renovation work involving facilities containing asbestos.
 - 8.2.3.5 Procedures for interim control measures and extraordinary precautions.
 - 8.2.3.6 Explanation of applicable OSHA and EPA rules.

8.3 Pre-Construction Asbestos Surveys (Pre-Construction Survey)

- 8.3.1 A pre-construction asbestos survey is performed in anticipation of renovation or demolition where a Baseline AHERA Survey has not been conducted and there is no information, or insufficient information, as to the existence of ACM within the

planned limits of construction. The Pre-Construction Survey requires destructive testing if concealed spaces are to be breached during construction. If ACM are found, a Project Design Asbestos Survey is conducted to provide information for preparing the plans and specifications. The Pre-Construction Asbestos Survey satisfies the EPA NESHAP requirements for renovation or demolition to “thoroughly inspect the affected facility” or the requirements of governmental agencies for issuance of a building permit. NOTE: ASTM has published detailed guidance on how to conduct a comprehensive building asbestos survey; contact ASTM for more information.

8.4 POLICY

- 8.4.1 It is VHA policy that all VHA facilities will comply with applicable EPA, Occupational Safety and Health Administration (OSHA), state, and local asbestos regulations for the safe management of ACM.

9 Reporting Requirements

9.1 Progress Reports

- 9.1.1 The contractor shall provide the COR with monthly written progress reports. These are due to the COR by the second workday following the end of each calendar month, throughout the project's duration.
- 9.1.2 The progress report shall cover all work completed during the preceding month and shall present the work to be accomplished during the subsequent month. This report shall also identify any problems that arose, along with a statement explaining how the problem was resolved.

9.2 Progress Meetings

- 9.2.1 Progress meetings shall be held to discuss progress of work in accordance with the schedule and quality control requirements.
- 9.2.2 Contractor and COR shall agree to date and time as well as re-occurrence of the progress meetings.
- 9.2.3 Contractor shall lead and record meeting minutes for all progress meetings.

10 Government Furnished Information

As referenced in FAR 52.236-4 and VAAR 852.236-71 of the solicitation all available record drawings will be provided upon request. They should not however, be considered “as-built”.

These drawing files are provided without warranty or obligation as to the accuracy or information contained in the files. All information in the files shall be independently verified by the user. Any user shall agree to indemnify and hold the VA harmless from any and all claims, damages, losses, and expenses, including, but not limited to, attorney fees arising out of the use of the Computer Aided Design and Drafting (CADD) drawing files.

11 Resources

11.1 VA Standards and Publications

All designs shall comply with applicable sections of VA standards and publications, and all references therein. Ensure that construction documents comply with the latest edition of these VA construction and design standards. VA standards can be found on the World Wide Web at <http://www.cfm.va.gov/TIL/>.

12 Confidentiality and Nondisclosure

- 12.1 It is agreed that the preliminary and final deliverables, and all associated working papers, application source code, and other material deemed relevant by VA which have been generated by the contractor in the performance of this task order, are the exclusive property of the U.S. Government and shall be submitted to the CO at the conclusion of the task order.
- 12.2 The CO will be the sole authorized official to release, verbally or in writing, any data, draft deliverables, final deliverables, or any other written or printed materials pertaining to this task order. No information shall be released by the contractor. Any request for information relating to this task order, presented to the contractor, shall be submitted to the CO for response.
- 12.3 Press releases, marketing material, or any other printed or electronic documentation related to this project, shall not be publicized without the written approval of the CO.

Appendix A – Drawings